

The Global Burden of Mental Illness



Prevalence, Impact, and the Treatment Gap

Global Data • 2023 | Source: Our World in Data / IHME, Global Burden of Disease

For: Public Health Policy Analysts & Mental Health Program Planners

Hundreds of Millions Affected

1 in 3 Women and 1 in 5 Men Will Experience Major Depression in Their Lifetime

1 in 3

W O M E N

will experience major depression
in their lifetime

1 in 5

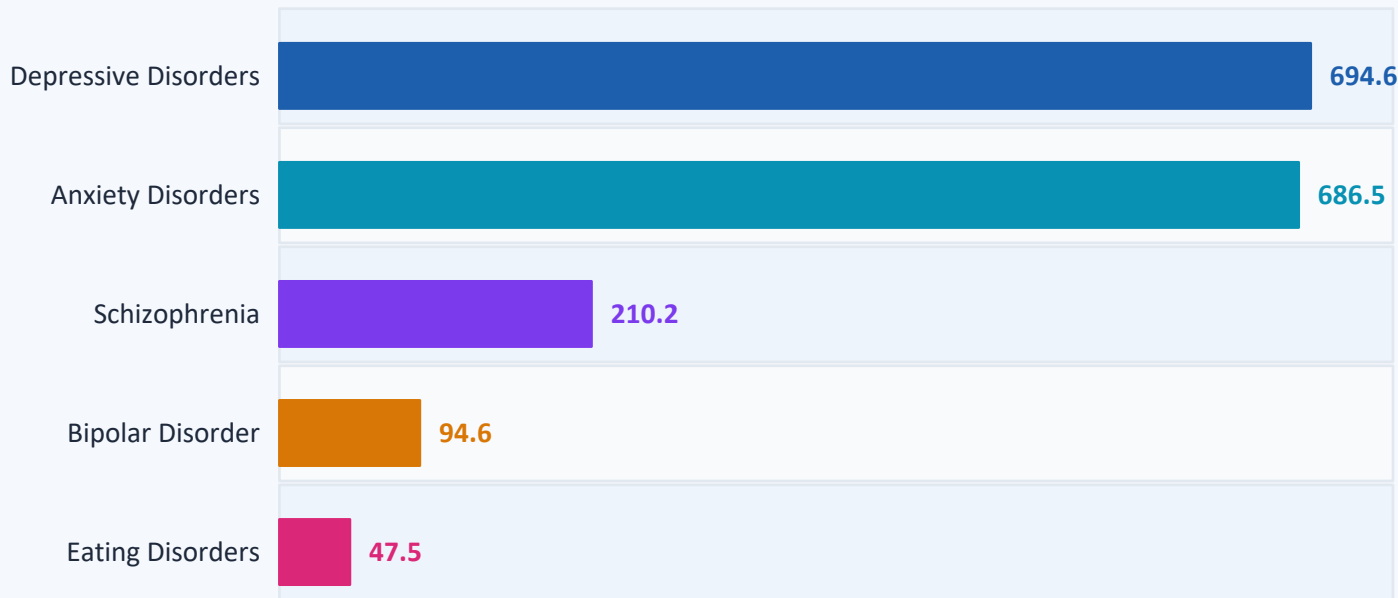
M E N

will experience major depression
in their lifetime

Beyond depression, **hundreds of millions** of people suffer from mental health conditions each year. Conditions span a spectrum — from common high-prevalence disorders (depression, anxiety) to less prevalent but severely disabling conditions (schizophrenia, bipolar disorder). Mental illnesses are **treatable**, yet treatment often remains lacking or of poor quality.

Depression & Anxiety Dominate Global Mental Health Burden

DALYs per 100,000 people, World, 2023



DALYs per 100,000 population

Key Insight: Depression and anxiety together account for ~80% of the total mental health disease burden across these five categories.

Depression and Anxiety Carry Nearly Equal Disease Burden Worldwide

DEPRESSIVE DISORDERS

694.6

DALYs per 100,000

World, 2023

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ANXIETY DISORDERS

686.5

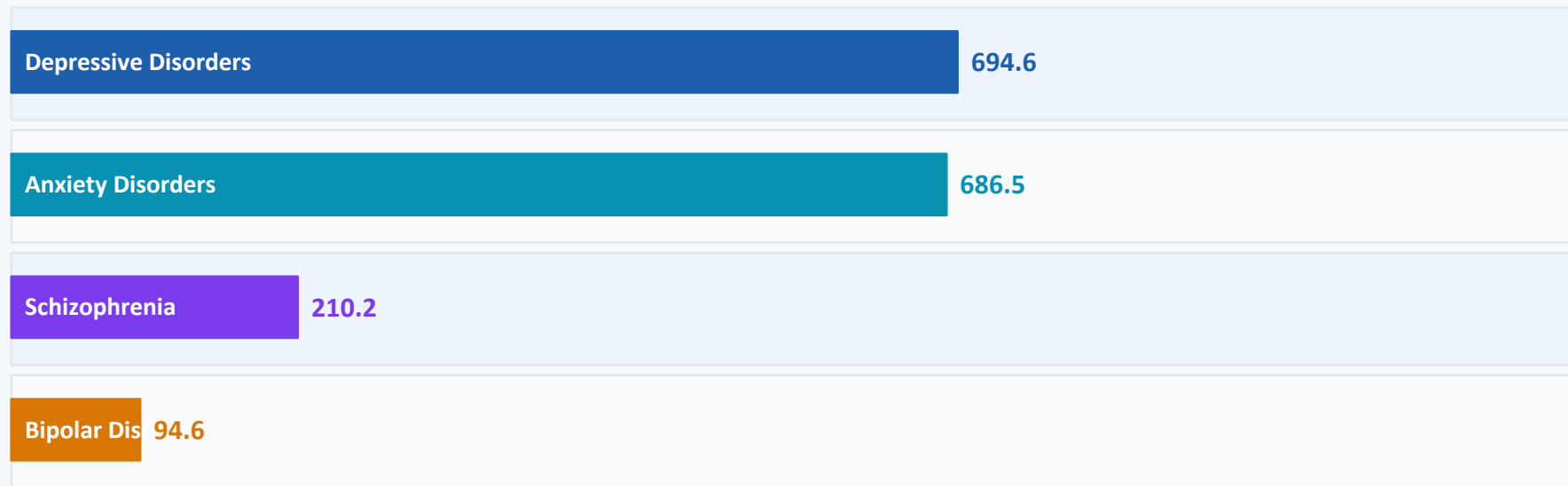
DALYs per 100,000

World, 2023

Difference of only 8.1 DALYs per 100,000 — effectively equal in scale. Together they represent the overwhelming majority of the global mental health disease burden, dwarfing all other categories combined.

Schizophrenia & Bipolar: Lower Prevalence, Severe Individual Impact

Proportional comparison — DALYs per 100,000, World, 2023



Schizophrenia contributes 210.2 DALYs/100,000 — 3× lower than depression in population terms, yet represents **severe disability at the individual level**. Bipolar disorder (94.6) similarly carries a disproportionate burden relative to its prevalence. Policy must maintain specialized care for these conditions even as resources scale toward high-prevalence disorders.

Treatment Is Available — But Often Lacking or Poor Quality



Stigma

Many people are uncomfortable sharing symptoms with healthcare professionals or people they know, suppressing help-seeking.



Access

Treatment is often lacking, especially in low- and middle-income countries where mental health infrastructure is limited.



Quality of Care

Even where treatment exists, it may be of poor quality — effective interventions are not always delivered consistently.



Underreporting

Discomfort with disclosure makes prevalence hard to estimate. The true burden is likely higher than reported figures suggest.

Good data is essential — to understand these conditions, how many people are affected, and how they can be treated effectively.

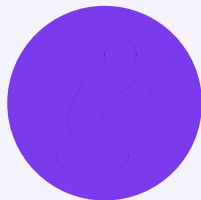
How People Cope: From Medication to Faith to Family Conversations

Global survey data reveals wide variation in how people deal with anxiety or depression



Prescribed Medication

Common in higher-income settings where healthcare systems provide structured pharmacological treatment.



Religious & Spiritual Activities

Prevalent across many regions worldwide, particularly in communities where faith plays a central role in wellbeing.



Talking to Friends or Family

A widely reported coping mechanism globally — informal social support as a primary mental health resource.

Policy implication: The diversity of coping strategies — shaped by cultural, economic, and healthcare system factors — means interventions must be **culturally sensitive** rather than one-size-fits-all.

Key Takeaways & Policy Implications

Prioritizing Depression, Anxiety, and Closing the Treatment Gap

01

Depression & anxiety represent the overwhelming majority of mental health DALYs (~695 and ~687 per 100,000 respectively) — policy investment should match this scale.

02

Schizophrenia and bipolar carry disproportionately high individual burden (210.2 and 94.6 DALYs/100K) despite lower prevalence — specialized care must be maintained.

03

Treatment exists but access and quality remain inadequate — particularly in low- and middle-income countries where infrastructure lags far behind need.

04

Stigma suppresses help-seeking and accurate data collection, meaning the true burden is likely underestimated. Destigmatization is a prerequisite for progress.

05

Scale evidence-based interventions for common mental disorders (depression, anxiety) while protecting specialized care for severe conditions. Culturally sensitive delivery is essential.